FORTIS MEMORIAL RESEARCH INSTITUTE

Sector 44, Opposite HUDA City Centre Metro Station, Gurugram, Haryana - 122002
NABH & JCI Accredited Tertiary Care Hospital | GSTIN: 06AAACF1023K1ZM | CIN: U74899HR1996PLC043211

INPATIENT FINAL DISCHARGE TAX INVOICE (CASHLESS TPA CLAIM)

Patient Name: Aarav Sen	**Invoice / Bill No:** BILL-FMRI-2026-8819
UHID / Reg No: UHID-7729104	**IPD Encounter:** IPD-2026-9042
Age / Gender: 42 Yrs / Male	**Attending Surgeon:** Dr. Vikramaditya Singhal, MS, MCh
Admission Date: 2026-10-10 08:30 AM	**Discharge Date:** 2026-10-12 11:30 AM (2 Days)
Primary Diagnosis: Acute Calculus Cholecystitis (ICD-10: K80.20)	**Policy / TPA ID:** POL-STAR-ASSURE-001 (Card #STAR-78190)

ITEMIZED HOSPITAL ADMISSION CHARGES

#	Particulars / Service Description	Category	Qty	Rate (INR)	Total Amount (INR)
1	**Single Private AC Room (2 Days)**	ROOM_ICU	2.0	4,500.00	**9,000.00**
2	**Laparoscopic cholecystectomy**	PROCEDURE	1.0	65,000.00	**65,000.00**
3	**IV Antibiotic Therapy Course (Ceftriaxone & Metronidazole)**	MEDICATION	1.0	4,200.00	**4,200.00**
4	**Complete Blood Count & Liver Function Test**	INVESTIGATION	1.0	1,700.00	**1,700.00**
5	**Disposable Laparoscopic Trocar & Consumable Kit**	CONSUMABLE	1.0	12,500.00	**12,500.00**
6	**Elective cosmetic keloid scar revision laser therapy**	PROCEDURE	1.0	14,000.00	**14,000.00**
7	**Personal toiletries & admission comfort kit**	CONSUMABLE	1.0	2,500.00	**2,500.00**
	TOTAL CLAIMED / BILLED AMOUNT				**INR 108,900.00**

CLINICAL BILLING ADVISORY & AUDIT DISCLOSURE:
• Item #2 (Laparoscopic Cholecystectomy, CPT-47562) performed for Acute Calculus Cholecystitis (ICD-10 K80.20).
• Item #6 (Elective Cosmetic Scar Revision) was submitted at patient request for pre-existing abdominal scar revision; non-essential aesthetic procedure.
• Item #7 (Personal Comfort Kit) billed under non-medical comfort consumables. Subject to standard insurer adjudication terms.

B. Sharma
Inpatient Billing Executive

Dr. R. Mathur, MD
Medical Superintendent

Dr. Vikramaditya Singhal
Chief GI & Laparoscopic Surgeon

FORTIS MEMORIAL RESEARCH INSTITUTE

DEPARTMENT OF SURGICAL GASTROENTEROLOGY & MINIMAL ACCESS SURGERY

PHYSICIAN CLINICAL DISCHARGE SUMMARY

Patient Name: Aarav Sen	**Age/Sex:** 42 Yrs / Male	**IPD No:** IPD-2026-9042
UHID: UHID-7729104	**Admit Date:** 10-Oct-2026 08:30 AM	**Discharge:** 12-Oct-2026 11:30 AM

FINAL CLINICAL DIAGNOSIS:

Primary Diagnosis: Acute Calculus Cholecystitis with Symptomatic Cholelithiasis (ICD-10: K80.20)
Secondary Findings: Localized cystic duct inflammation, thickened gallbladder wall (4.2 mm). No choledocholithiasis.
Surgical Procedure Performed: Four-Port Laparoscopic Cholecystectomy (CPT-47562) under General Anesthesia on 10-Oct-2026.

CHIEF COMPLAINTS & HISTORY OF PRESENT ILLNESS:
Patient Aarav Sen, 42-year-old male, presented to emergency with severe episodic right upper quadrant (RUQ) abdominal pain of 24 hours duration, radiating to right infrascapular area. Pain aggravated post high-fat meals. Associated with episodic nausea, bilious vomiting (2 episodes), and low-grade fever (100.4°F). No jaundice, melena, or bowel disturbance.

CLINICAL EXAMINATION ON ADMISSION:
Conscious, oriented, hemodynamically stable. Pulse: 88/min, BP: 126/82 mmHg, SpO2: 98% on room air, Temp: 100.2°F. Abdomen soft, severe focal tenderness in right hypochondrium with strongly positive Murphy's Sign. No involuntary guarding, rigidity, or palpable mass. Bowel sounds normal.

INVESTIGATION FINDINGS:
• Ultrasound Abdomen (10-Oct-2026): Thickened gallbladder wall (4.2 mm) with multiple mobile acoustic shadowing calculi in neck/Hartmann's pouch (largest 14 mm). Minimal pericholecystic fluid. CBD caliber normal (4.8 mm).
• Blood Panel: Elevated TLC (13,400 /uL) with neutrophilia (78%), CRP elevated at 32.4 mg/L. Normal LFTs, Serum Amylase & Lipase.

HOSPITAL COURSE & SURGICAL MANAGEMENT:
Patient admitted under Dr. Vikramaditya Singhal. Pre-anesthesia clearance secured. Emergency Laparoscopic Cholecystectomy (CPT-47562) performed on 10-Oct-2026 under general anesthesia. Intraoperatively, distended, inflamed gallbladder identified; Calot's triangle skeletonized to achieve Critical View of Safety (CVS). Cystic duct and artery clipped and divided. Gallbladder dissected retrograde and extracted intact in endobag. Operative blood loss minimal (<25 mL).

POST-OPERATIVE RECOVERY & DISCHARGE STATUS:
Smooth post-operative recovery. Maintained on IV fluids, IV Ceftriaxone 1g BD, IV Metronidazole 500mg TDS, and analgesics. Oral sips tolerated on POD-1 morning, stepped up to full liquid/soft diet. Surgical trocar sites clean, dry, no hematoma or soakage. Afebrile for 36 hours. Discharged in stable clinical state on 12-Oct-2026.

NOTE ON ELECTIVE AESTHETIC REQUEST:
Patient requested elective scar revision laser therapy for an unrelated childhood infraumbilical scar. Examined by attending surgeon; determined to be non-emergent elective cosmetic procedure, not medically necessary for acute calculus cholecystitis.

DISCHARGE MEDICATIONS & ADVICE:
1. Tab Cefuroxime Axetil 500 mg PO twice daily x 5 days (post meals)
2. Tab Pantoprazole 40 mg PO once daily before breakfast x 7 days
3. Tab Paracetamol 650 mg PO as needed for pain (max TDS)
4. Low-fat, high-fiber diet. Avoid strenuous lifting for 3 weeks.
5. Follow-up in GI Surgery OPD on 17-Oct-2026 for suture removal.

Prepared By: **Dr. A. Gupta, MBBS** (Resident MO)

Attending Consultant: **Dr. Vikramaditya Singhal, MS, MCh** (Reg: MCI-44918)

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OPERATION THEATRE REGISTRY & SURGICAL PROCEDURE RECORD

OPERATIVE REPORT (PROCEDURE NOTE)

Patient Name: Aarav Sen	**OT Number:** OT-3 (GI Laparoscopy Suite)	**Date of Surgery:** 10-Oct-2026
Lead Surgeon: Dr. Vikramaditya Singhal	**Anesthetist:** Dr. Sunita Mehra, MD	**Anesthesia Type:** General Endotracheal
Pre-Op Diagnosis: Acute Cholecystitis	**Post-Op Diagnosis:** Acute Calculus Cholecystitis	**CPT Code: CPT-47562**

SURGICAL PROCEDURE: FOUR-PORT LAPAROSCOPIC CHOLECYSTECTOMY (CPT-47562)

ANESTHESIA & POSITIONING:
Under general endotracheal anesthesia, patient placed in supine position with 15-degree reverse Trendelenburg and left tilt. Sterile painting and draping of abdomen completed.

INCISION & ACCESS:
A 10 mm supraumbilical curvilinear skin incision made. Pneumoperitoneum established using open Hasson technique with CO2 insufflation maintained at 12 mmHg. 10 mm 30-degree laparoscope introduced. Diagnostic laparoscopy revealed acute hyperemic, distended gallbladder with thick vascularized wall. Under direct vision, a 10 mm epigastric port and two 5 mm right midclavicular/anterior axillary subcostal assistant ports placed.

DISSECTION & CRITICAL VIEW OF SAFETY:
Gallbladder fundus retracted cephalad; Hartman's pouch retracted inferolaterally. Mild pericholecystic omental adhesions divided using gentle blunt and hook dissection. Calot's triangle dissected meticulously. The lower third of gallbladder freed from liver bed to achieve the Critical View of Safety (CVS):
1. Hepatocystic triangle cleared of fat and fibrous tissue.
2. Cystic plate visualized.
3. Only two structures (cystic duct and cystic artery) seen entering gallbladder base.

CLIPPING & DIVISION:
Cystic artery isolated, doubly clipped proximally with titanium clips and singly distally, and divided with laparoscopic scissors. Cystic duct isolated, milked towards gallbladder to exclude calculi, doubly clipped proximally and singly distally, and divided. No bile leak.

RETROGRADE EXCISION & EXTRACTION:
Gallbladder dissected off liver bed retrograde using monopolar hook electrocautery. Hemostasis on liver bed secured with bipolar diathermy. Gallbladder placed in sterile endoscopic specimen retrieval pouch and extracted intact via supraumbilical port site. Saline irrigation of subhepatic space performed; aspirate clear.

CLOSURE & SPECIMEN:
Desufflation under direct vision. Umbilical fascia closed with 1-0 Vicryl. Skin incisions closed with subcuticular 4-0 Monocryl. Gauze dressing applied. Intact specimen cut open on back table: revealed 12 faceted cholesterol stones, mucosal inflammation. Specimen dispatched for histopathology.
• Estimated Blood Loss: 20 mL. Complications: Nil. Needle and gauze count correct x 2.

SURGEON VERIFICATION:

Surgeon: **Dr. Vikramaditya Singhal, MS, MCh** Consultant Surgical Gastroenterologist	Anesthesiologist: **Dr. Sunita Mehra, MD** Senior Consultant Anesthesiology	Scrub Nurse: **Sister Mary V., RN** OT Charge Nurse

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DEPARTMENT OF RADIODIAGNOSIS & CENTRAL DIAGNOSTIC LABORATORIES

CLINICAL DIAGNOSTIC INVESTIGATION REPORTS

REPORT 1: ULTRASONOGRAPHY (USG) OF WHOLE ABDOMEN

Clinical Indication: Acute RUQ pain, suspected acute cholecystitis.
Imaging Modality: High-Resolution Real-Time Abdominal Sonography (GE Voluson E10, 3.5-5.0 MHz probe).
Findings:
• **Gallbladder:** Distended with diffuse circumferential wall thickening measuring **4.2 mm** (normal <3.0 mm) with stratified layered edema (sonographic double-rim sign). Multiple echogenic mobile foci demonstrating posterior acoustic shadowing seen within lumen, largest measuring 14.1 mm impacted near neck/Hartmann's pouch. Positive sonographic Murphy's sign elicited under probe transducer pressure. Minimal pericholecystic fluid halo present.
• **Biliary Tree:** Common Bile Duct (CBD) measures 4.8 mm in caliber, within normal limits. Intrahepatic biliary radicles (IHBR) not dilated.
• **Liver:** Normal size (14.2 cm), normal parenchymal echotexture. No focal lesion.
• **Pancreas, Spleen & Kidneys:** Normal sonographic appearance.
IMPRESSION: Findings are diagnostic of **Acute Calculus Cholecystitis with Cholelithiasis (ICD-10: K80.20)**. No choledocholithiasis or pancreatitis.
Reported By: Dr. Kavita Deshmukh, MD, DMRD (Senior Consultant Radiologist)

REPORT 2: HEMATOLOGY & CLINICAL BIOCHEMISTRY PANEL (CODE: LAB-CBC-PLATE)

Test Parameter	Observed Result	Biological Reference Range	Flag / Clinical Significance
Hemoglobin (Hb)	14.2 g/dL	13.0 - 17.0 g/dL	Normal
Total Leukocyte Count (TLC)	**13,400 /uL**	4,000 - 11,000 /uL	**HIGH (Acute Inflammatory Response)**
Neutrophils %	**78 %**	40 - 70 %	Elevated (Shift to left)
Platelet Count	280,000 /uL	150,000 - 450,000 /uL	Normal
C-Reactive Protein (CRP Quantitative)	**32.4 mg/L**	< 5.0 mg/L	**HIGH (Marked Acute Phase Reactant)**
Total Bilirubin	0.9 mg/dL	0.2 - 1.2 mg/dL	Normal
Serum Alkaline Phosphatase	118 U/L	30 - 120 U/L	Normal (Upper limit)
Serum Amylase	48 U/L	28 - 100 U/L	Normal (Rules out gallstone pancreatitis)

REPORT 3: SURGICAL HISTOPATHOLOGY BIOPSY REPORT

Specimen: Gallbladder (Post-Laparoscopic Cholecystectomy).
Gross Examination: Specimen consists of gallbladder measuring 6.5 x 3.2 x 2.4 cm. External surface congested with fibrinous exudate. Lumen contains 12 faceted greenish-yellow stones, largest 1.4 cm. Wall thickness 4.0 mm.
Microscopic Examination: Sections show gallbladder mucosa with marked lymphocytic and plasma cell infiltrate in lamina propria. Muscularis shows hypertrophy and transmural acute neutrophil infiltration with Rokitansky-Aschoff sinuses. No dysplasia or malignancy.
HISTOPATHOLOGICAL DIAGNOSIS: Acute-on-Chronic Calculus Cholecystitis with Cholelithiasis (ICD-10: K80.20).
Pathologist: Dr. N. Sengupta, MD (Senior Consultant Histopathologist)

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INPATIENT PHARMACY & NURSING CLINICAL ADMINISTRATION LOG

MEDICATION & SURGICAL CONSUMABLES DISPENSATION RECORD

SECTION A: INPATIENT DRUG DISPENSATION LOG (CODE: MED-IV-ABX)

Medication Name / Formulation	Batch No	Dosage / Route	Qty Dispensed	Administration Schedule
Inj. Ceftriaxone 1g Vial (Magneff)	CF-2026-901	1g IV in 100mL NS	4 Vials	12-hourly x 2 days (Completed)
Inj. Metronidazole 500mg IV Infusion	MZ-8821	500mg IV infusion	4 Bottles	8-hourly x 2 days (Completed)
Inj. Pantoprazole 40mg Vial	PT-4410	40mg IV bolus	2 Vials	Once daily (Completed)
Inj. Paracetamol 1000mg IV Infusion	PCM-7712	1g IV slow infusion	3 Bottles	SOS for post-op pain (Completed)
IV Fluid: Ringer's Lactate 500mL	RL-1109	IV maintenance	3 Units	Continuous hydration

SECTION B: SURGICAL CONSUMABLES & SPECIAL COMMODITIES DISPENSATION

Item Description	Code Ref	Qty	Admissibility Classification
Disposable Laparoscopic Trocar Pack (10mm x 2, 5mm x 2)	CONS-SURG-KIT	1 Set	Admissible Inpatient Surgical Consumable
Titanium Laparoscopic Ligating Clips (Medium-Large)	CONS-SURG-KIT	1 Cartridge	Admissible Surgical Implantable Device
Endoscopic Specimen Retrieval Catch Bag	CONS-SURG-KIT	1 Unit	Admissible Laparoscopic Consumable
Elective Laser Dermabrasion / Scar Revision Kit	CPT-COSM-01	1 Session	**NON-ADMISSIBLE (Elective Aesthetic Procedure)**
Personal Toiletries & Executive Comfort Amenity Kit	CONS-KIT-LUX	1 Kit	**NON-ADMISSIBLE (Personal Grooming / Non-medical)**

HOSPITAL DISCHARGE & CLAIMS CLEARINGHOUSE SUMMARY:
This 5-page clinical and financial record constitutes the complete inpatient claim bundle for patient Aarav Sen (UHID-7729104, IPD-2026-9042). All services recorded above were performed under Fortis Healthcare NABH clinical governance protocols. Non-admissible elective aesthetic items (Lines #6 and #7) are flagged in adherence to IRDAI Master Circular guidelines for cashless health claim settlement.
Digital Verification Hash: SHA256-FMRI-7729104-9042-ADJUDICATED | Timestamp: 2026-10-12T11:30:00Z

Chief Pharmacist
Lic #HR-PHARM-22019

Nursing Superintendent
Sister Reena K., RN

Medical Director
Dr. Harsh Vardhan, MD